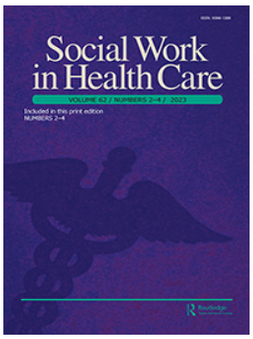

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Fintech and medical expenses: What social workers should know

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ABSTRACT

Individuals who cannot afford out of pocket medical expenses may reduce health care use, resulting in poorer health outcomes. To ease the situation, employers turn to financial technology ("fintech") health care credit applications. We examine whether an employer-sponsored credit fintech application (MedPut) helps employees manage medical expenses. Results of the analysis of variance (ANOVA) and probit regression models reveal MedPut users did worse financially and delayed health care due to cost more often than employees who did not use MedPut. Results may inform social work policy and direct practice perspectives on fin-tech and medical expenses.

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Medical debt; financial technology; out-of-pocket costs; medical credit; health care use

The United States (U.S.) has a unique system of financing health care in which employers, employees, other individuals, and government pay into various public and private insurance plans that finance care. Over half of individuals have health insurance through their employer. Plans offered by employers vary widely concerning employees' out-of-pocket (OOP) expenses including premiums, co-payments, deductibles, and co-insurance. More than half (59%) of all employees in the U.S. are covered by a high deductible health plan (HDHP) through their employer (Kaiser Family Foundation, 2022a). These plans are attractive to employers because premiums are typically lower than for other forms of employer-sponsored insurance. HDHPs require that employees incur OOP up to the deductible amount at least $1,400 for an individual and $2,800 for a family (HealthCare.gov, 2022) before the plan kicks in, unless their employer also funds health reimbursement accounts (HRAs) or health savings accounts (HSAs).

Lower-paid employees enrolled in HDHPs without employer HRA or HSA contributions might find it hard to come up with the cash to pay OOP health care expenses until they reach their deductible (Bernstein, 2014). This

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challenge is even greater among employees who already have unpaid health-care bills. Recognizing these challenges, some employers have offered employees MedPut, a digital financial application that helps employees pay medical bills and other health care costs using interest-free payment plans and bill negotiation services. The purpose of this study was to examine the registration choice, use of, and benefits from using MedPut among employees enrolled in HDHPs. Accordingly, our research questions are as follows:

(1) What were the registration and usage rates for MedPut? Did these rates vary based on employee and employer characteristics?
(2) What were the frequency, amount, and disposition of health care bills submitted for payment? Did bill submission vary based on employee characteristics?
(3) What were the experiences of employees in paying for health care? To what degree do employees put off health care due to cost concerns? Did these experiences differ among MedPut users and non-users?

Findings from this study may inform employer perspectives on health insurance and other benefits and policy concerning the intersection of health care financing, consumer financial health, and health outcomes. Social workers may advocate to improve employer health insurance policy and health care financing options to better protect financially vulnerable populations.

Review of the literature

Prior research draws a connection between individuals' OOP expenses and their health care use (Al Rowas et al., 2017; Brot-Goldberg et al., 2017; Wharam et al., 2018). These studies found that individuals are sensitive to changes in OOP expenses and reduce health care consumption when these expenses increase. Another area of research draws a link between medical debt and lessened health care use (Bennett & Dismuke, 2010; Doty et al., 2005; Wiltshire et al., 2016; Zhang et al., 2018). When confronted with high deductibles and existing medical debt, individuals' reductions in health care use may have downstream negative effects; they may avoid preventive care or put off receiving care for a chronic illness and risk experiencing a catastrophic and expensive care event.

Difficulties affording OOP healthcare expenses may result in accumulating medical debt; Himmelstein et al. (2019) found that nearly 60% of individuals filing for bankruptcy cited medical expenses as a key reason. Difficulty repaying medical debt may render individuals reluctant to seek additional care because it would add to an already unmanageable debt burden (Doty et al., 2005).

Individuals have an array of choices: personal savings, HRAs (if offered by their employer), HSAs, Flexible Spending Accounts (FSAs), credit cards, unsecured bank loans, hospital and health care provider charity care discounts and payment plans, and payday loans and similar high-interest rate credit products. Yet Seymour (2020) found that most individuals with cancer used their personal savings to cover medical bills but were still unable to avoid medical debt. Peter et al. (2016) found that individuals who used a HDHP paired with an HSA saved more but reduced their use of health care services compared to respondents who were enrolled in a full coverage health care plan. Kalousova and Burgard (2013) found a positive association between credit card debt, medical debt and delaying health care services regardless of socioeconomic and health differences. These findings suggest that as debt rises, health care use falls.

The focus of our study is on a fintech product meant to help workers in HDHPs better afford healthcare. Fintech offers access to new types of financial products and services that may benefit individuals and families. Digital and mobile payment platforms make it easier for people to conduct transactions and send and receive money from family members and friends, personal financial management apps make it easier to track spending, and savings apps can help people build emergency savings via goal setting, transaction round-ups, and automatic saving (Despard, in press). A review of over 60 studies of fintech showed that the use of fintech is correlated with improvements in spending, loan repayment, and saving behaviors and can improve access to credit, yet overall evidence on the benefits of fintech is mixed and it is particularly unclear whether fintech benefits lower-income individuals and families (Despard, in press). In our study, MedPut may have given workers an affordable source of credit to pay overdue medical bills and some relief through bill reductions. A fintech credit product like MedPut is especially important for workers who have low or no credit scores and cannot access affordable credit (Atkinson, 2020; Urban Institute, 2021). In describing results from our study of MedPut, we can help shed light on ways in which fintech may help workers address problems paying for and utilizing healthcare.

Methods

This study was part of a project called the Financial Solutions Lab (FSL) led by the Financial Health Network, a national nonprofit. The Lab tests different ways to promote financial health using digital technologies. We partnered with the Financial Health Network to study digital financial products and services offered through the workplace as part of the 5th cohort of the Lab.

For this study, we examined one of these products, MedPut, a digital financial application that helps employees pay medical bills and other health care costs using interest-free payment plans and bill negotiation services. The

monthly fee to have MedPut is paid by the employer, the employee, or a combination of the two. The employer-paid fee is $2 to $6 per month per registered employee, depending on the financing limit, which is from $1,000 to $3,000. The employee-paid fee is $3 to $7 per month. MedPut focuses on employees enrolled in HDHPs who may need help paying for health care expenses until they meet their deductible. Employees register to use the MedPut app and upload the health care bills they need help paying. MedPut pays the bills after trying to negotiate discounts for the employee. Employees make monthly payments via payroll deductions. The amount that employees can borrow is limited by monthly repayments capped at 5% of gross pay. Repayment terms vary from one month to two years.

Data and sample

Data for this study came from two sources. First, administrative data from the MedPut platform was de-identified and shared with researchers for analysis. Data were included for 4,317 employees in 21 companies and organizations who were invited to use MedPut from January 2018 through July 2020. Data included employees' age, salary range, employer size and industry, and health care bills submitted for payment. Second, employees who were registered for MedPut were invited by MedPut to complete a survey on two occasions, including 120 employees who completed a survey in November 2019 and 104 employees who completed the same survey in June 2020. A total of 35 employees completed both surveys. Survey questions asked about employees' experiences with medical bills and debt, use of health care, general household financial health, and demographic characteristics. Because MedPut provided de-identified administrative and survey data to the authors for analysis, the Institutional Review Board at a university in the Midwest determined the project to be not human subjects research.

Variables

Dependent variables included whether employees registered for the MedPut app, whether registered employees used the app at least once, the number, amount, and disposition of submitted bills – current, delinquent, or advance payment, and whether a discount was negotiated and the amount of this discount. These dependent variables were measured using MedPut administrative data. Independent variables measured from MedPut administrative data included age, job tenure, salary, fee type (self, employer, or shared fee), employer size, employer industry, and any dependents in the home.

Additional dependent variables were measured from a survey that MedPut users completed concerning financial difficulties and use of MedPut. Financial difficulty variables included having problems paying medical bills and being

contacted by a collections agency within the prior 12 months, and ability to cover household expenses. Also, participants were asked whether in the prior six months they had lost a job or taken a pay cut due to illness or injury, postponed getting medical or dental care, and/or postponed filling a prescription medication because they could not afford it.

Analysis

To answer Research Question 1 concerning variation in MedPut registration and use, we estimated the marginal effects of and predicted probabilities associated with independent variables on the registration choice using probit regression models with covariance control, robust standard errors, and employer as a cluster variable. Covariates used in probit models included age, salary group, any dependents in the home, MedPut fee type, industry, and employer size. To answer Research Questions 2 and 3, we used univariate descriptive statistics and bivariate analysis including chi square tests for categorical and ordinal level independent variables (e.g., salary group, industry) and t-tests and analysis of variance (ANOVA) for age and job tenure.

Results

Sample description

The mean age of employees who had access to MedPut was 42.79 ($SD = 12.69$). Most (73.20%) employees worked for a large company and the most common type of industry was white collar. Over a third of employees had salaries below $40,000 and had dependents. Out of 4,317 employees who were invited, 1,234 or 28.5% registered for MedPut which meant they could submit health care bills for payment. Among registrants ($N = 1{,}234$), 21.31% used MedPut at least once during the study period of 2.5 years. Table 1 provides a full description of the sample.

Differences in MedPut registration and use

Marginal effects estimated from probit regression predicting MedPut registration indicated that employees with dependents were more than three times as likely to register for MedPut (54.0%) compared to employees without dependents (13.8%) ($p < .01$). Employees in small companies or organizations were twice as likely to register (68.5%) as employees in medium (29.8%) and large (22.5%) companies or organizations ($p < .001$). Other factors that were statistically significant predictors of MedPut registration included lesser job tenure, when employees paid the full fee for MedPut, and employees working in blue collar industries. Details of the health

Table 1. Sample description and MedPut status.

	Mean (*SD*) or %			
	Total	Users	Registered	Invited
Age	42.79(12.69)	43.31 (10.79)	42.99 (12.01)	42.68 (13.06)
Tenure (mos.)*	80.16 (90.18)	76.44 (71.89)	82.45 (92.71)	76.44 (71.89)
Salary***				
<$40k	35.49	26.62	32.03	37.33
$40 – 69k	40.63	50.57	43.36	38.92
$70k+	23.88	22.81	24.61	23.74
Any dependents***	34.75	69.58	58.39	24.33
Fee Type***				
Employer paid	27.33	21.67	24.10	28.84
Employee paid	34.31	52.47	52.11	27.15
Shared	38.36	25.86	23.79	44.02
Employer size**				
Small	3.71	5.70	5.25	3.05
Medium	23.09	20.91	21.42	23.81
Large	73.20	73.28	73.33	73.14
Industry type***				
Blue-collar[a]	20.64	17.49	17.40	21.93
White-collar[b]	52.98	48.67	47.06	55.21
Services	14.57	19.01	23.07	11.51
Government	11.81	14.83	12.46	11.35

$N = 4{,}317$ ($n = 263$ for users, $n = 971$ for registered, $n = 3{,}083$ for invited).
[a]Construction, manufacturing.
[b] Information technology, finance/insurance, professional, scientific, or technical services.
* $p < .05$, ** $p < .01$, *** $p < .001$.

Table 2. Health care and financial circumstances of MedPut users and non-users.

	Total %	Users %	Non-Users %
In the past 12 months:			
Problems paying a medical bill***	29.73	52.46	18.55
Contacted by collections agency***	28.25	45.76	19.49
Good financial situation[a]	70.49	62.30	74.59
In the past 6 months:			
Lost a job or had pay cut due to illness or injury	9.89	13.33	8.20
Put off medical or dental care due to cost***	42.86	63.33	32.79
Put off getting a prescription filled due to cost**	16.48	26.67	11.48

Note: $N = 185$ ($n = 61$ users and $n = 124$ non-users).
[a]Live comfortably or able to meet basic expenses with a little left over for extras.
* $p < .05$, ** $p < .01$, *** $p < .001$.

care financing circumstances of MedPut users and non-uses is provided in Table 2.

Marginal effects estimated from probit regression predicting usage among employees who registered for MedPut, indicated that employees with dependents were more likely to use MedPut (23.7%) compared to employees without dependents (15.8%) ($p < .05$). Employees in blue collar industries were more likely to use MedPut (32.4%) than employees in the service sector (15.9%) ($p < .05$), while employees who paid the full fee for MedPut were more likely to use it (25.7%) than employees whose

Table 3. Marginal effects: MedPut usage rate.

	dy/dx	SE	|z|	*p*
Salary group (REF: Low <$40k)	.022	.042	.51	.609
Middle ($40-69k)	-.030	.046	.65	.513
High ($70k+)				
Age	-.001	.001	.67	.505
Job tenure (years)	.000	.002	.04	.968
Any dependents in the home (REF: None)	.079	.032	2.49	.013
Fee type (REF: Employed paid)	.048	.039	1.21	.225
Shared fee	.124	.048	2.58	.010
Employee paid				
Industry type (REF: Blue collar)	-.145	.095	1.52	.128
White collar	-.165	.081	2.05	.040
Service sector	-.085	.086	.99	.325
Government				
Employer size (REF: Small)	-.063	.079	.80	.426
Medium	-.009	.067	.14	.887
Large				

Note: Results are from probit regression using marginal effects where dy/dx is the percentage point change in probability of the MedPut usage rate based on a one-unit change in the independent variable.

employer paid the full cost (13.3%) ($p < .05$). MedPut usage rates are listed in Table 3.

Number, disposition, and amount of medical bills submitted

Most employees (82%) who used MedPut did so only once; 15% used it twice, and the remaining 3% used it three or more times; employees submitted a total of 485 bills through 263 opened MedPut cases. Most cases[1] were for delinquent accounts (77%), followed by current bills (20%), and advance payments (3%) – such as when a provider requires an upfront deposit. Of the cases for delinquent accounts, 21% were for accounts in collections, which means the bill had been assigned or sold by the health care provider to a third party debt collection agency due to nonpayment. Results from regression models indicated that no factors (e.g., age) had a statistically significant relationship with total bills submitted or with bill disposition.

The mean amount of medical bills submitted through MedPut was $1,378.96 ($SD$ = $1,032.09) with a range of $44 to $4,958 and a median of $1,066.60. Results from a regression model predicting bill amount indicated that an increase in one year of age was associated with $15 more in submitted bills ($p < .05$), yet no other factors were statistically significant.

Negotiated bill reductions

Over a third (37%) of submitted bills were negotiated downward with health care providers or collection agencies, saving employees an average of $117 per case, with a range of $0 to $2,728. Among cases with successfully negotiated reductions, the average discount was 18%. Bills that were reduced were higher

in their original amount ($1,749) than bills that were not reduced ($1,163) ($p < .001$). Most health care providers willing to reduce the amount of money owed for care received do so on the basis of charity care program policies that consider patients' income. However, employees' salary range was unrelated to receiving a bill reduction nor was there a statistically significant difference in the likelihood a bill would be reduced based on whether it was current or delinquent. However, 69% of bills in collection were reduced compared to only 30% of bills not in collections ($p < .001$).

Employee health care experiences and financial circumstances

Employees who registered for MedPut and completed a survey ($N = 263$) gave responses concerning their experiences with medical bills, concerns about health care costs, and their financial circumstances. MedPut users were worse off financially and were more likely to have put off health care due to cost concerns compared to employees who registered for but did not use MedPut. Over half (52.46%) and nearly half (45.76%) of users said they had problems paying a medical bill and were contacted by a collections agency within the prior 12 months, compared to 18.55% and 19.49% of non-users ($p < .001$), respectively. Users were also more likely to put off medical or dental care (63.33%) and put off filling a prescription (26.67%) due to cost concerns compared to 32.79% ($p < .001$) and 11.48% ($p < .01$) of non-users, respectively. Employee health care experiences and financial circumstances are presented in Table 4.

Discussion

This study examined the characteristics and experiences of employees enrolled in HDHPs across 21 companies who were offered MedPut, which allows employees to borrow money to pay health care bills and repay amounts borrowed through automatic payroll deductions. Regarding our first research question, having dependents was a significant predictor of both MedPut

Table 4. Healthcare and financial circumstances of MedPut users and non-users.

	Total %	Users %	Non-Users %
In the past 12 months:	29.73	52.46	18.55
Problems paying a medical bill***	28.25	45.76	19.49
Contacted by collections agency***			
Good financial situation[a]	70.49	62.30	74.59
In the past 6 months:			
Lost a job or had pay cut due to illness or injury	9.89	13.33	8.20
Put off medical or dental care due to cost***	42.86	63.33	32.79
Put off getting a prescription filled due to cost**	16.48	26.67	11.48

Note: $N = 185$ ($n = 61$ users and $n = 124$ non-users).
[a]Live comfortably or able to meet basic expenses with a little left over for extras.
** $p < .01$, *** $p < .001$

registration and use. This finding suggests that employees with family coverage may have cash flow challenges in meeting their deductibles, as plan benefits for any family member do not begin until the full family deductible is met, with certain exceptions like annual physicals. We also found that employees in small companies were more likely to register for MedPut. The combination of these two findings may reflect that 31% of workers in small companies pay greater than 50% of family premiums compared to 7% among workers in larger companies (Kaiser Family Foundation, 2022b). Individuals are more likely to have medical debt if they have dependents. In addition, if premiums are taking up quite a bit of a person's income, they may experience a greater challenge in paying their medical debt.

In answering the second research question, the vast majority of bills submitted to be paid by MedPut were for delinquent accounts, a fifth of which were in collections status. This finding indicates that employees who used MedPut faced a double-bind of paying for health care out-of-pocket until they reached their deductible and dealing with existing medical debt. That over a third of bills were reduced through negotiation with health care providers and collection agencies suggests that employees may have more leverage than they realize in getting portions of owed amounts forgiven. Accounts in collection were more than twice as likely to be reduced than current and delinquent accounts.

Lastly, MedPut appeared to reach a group of employees who struggle with paying for health care and are prone to delay seeking health care because of cost concerns. This finding is important because prior research indicates that individuals with HDHPs – including those with chronic illnesses such as cancer – are more likely than those without HDHPs to delay seeking health care (Al Rowas et al., 2017; Wharam et al., 2018, Wharam et al., 2019). In turn, delayed care is associated with poor health outcomes (Kraft et al., 2009; Prentice & Pizer, 2007). Many financial products and services available to patients may not relieve them of medical debt or improve healthcare use rates. Several researchers demonstrated the negative financial and health outcomes patients experienced due to high OOP costs for health care services (Kalousova & Burgard, 2013; Peter et al., 2016; Seymour, 2020). Associations between medical debt and health care use reveal individuals who have more medical debt are more likely to avoid seeking health care services to the detriment of their health (Bennett & Dismuke, 2010; Doty et al., 2005; Wiltshire et al., 2016; Zhang et al., 2018).

Study strengths and limitations

This study is one of the few to examine the impact of a fintech application (MedPut) on employees' medical expenses. However, several limitations exist. First, only one cohort was included in this study and adding more cohorts may help to diversify the sample. Second, data was collected twice, however, only 35

employees completed both surveys. The low rate of survey completion limits our ability to conduct any longitudinal analyses. Future studies could offer an incentive to increase the rate of employee participation in the study over time. Third, no data on employee health status was collected, which may impact the internal validity of the study. Wharam et al. (2018) found that patients suffering from a chronic illness (macrovascular diabetes) were more likely to delay seeking health care because of costs than those who did not have a chronic illness. Similarly, employees in this study who have a chronic illness may be more likely to delay seeking health care due to costs than employees who do not have a chronic illness. Fourth, we did not have detailed information about employees' health plans. Our results may have been different had we accounted for elements of patient cost share such as deductible amounts and coinsurance rates.

Implications for practice, policy and research

Fintech may make it easier for people to access financial products and services that better meet their needs than traditional banking. MedPut provided a source of affordable credit to workers who may have lacked better options for getting caught up on their medical bills. Yet fintech cannot solve the financial problems individuals and families face due to low wages, unaffordable housing, healthcare, and childcare, and a lack of social protection (Friedline, 2020). For example, a popular fintech app in the workplace is earned wage access (EWA), where workers can access funds in advance of payday to help pay bills or meet other needs (Despard et al., 2020a). Yet EWA would mostly be unnecessary if employers paid living wages (Despard et al., 2020b) and workers had paid leave, universal childcare, affordable healthcare and other social protections.

From an employer perspective, the $2 to $6 monthly fee is a modest cost to offer MedPut as a benefit for employees enrolled in HDHPs to pay for healthcare while in their deductible periods. Yet we found that MedPut was used mostly to pay for overdue medical bills. More impactful ways for employers to help employees afford healthcare are to offer insurance plans with lower deductibles, offer wage-based premiums (i.e., lower-paid employees pay lower premiums) and fund employee HSAs for the deductible amount so employees can use money in the HSA to pay for care while under the deductible. But employers may be unwilling or unable to pay for these more expensive solutions, which suggests the need for changes in public policies concerning health coverage as we outline below.

That over a third of bills submitted to MedPut were reduced through negotiation suggests that hospitals and other health care providers could do a better job of offering charity care discounts (Velasquez, 2021) and zero-interest payment plans to patients, especially when patients with HDHPs are in their deductible periods. Nearly half of U.S. employees are low-wage earners whose median hourly wages fall well within income guidelines for most charity

care programs (Ross et al., 2020). Yet charity care accounts for less than 2% of expenses among hospitals (Zare et al., 2021). ACA provisions under Section 501(r)(4) of the Internal Revenue Code could be strengthened with respect to nonprofit hospitals' required reporting on Schedule H of the Form 990. Social workers in hospital settings are ideally situated to assist screening patients for financial assistance eligibility, especially for patients in need of specialty care which is harder for uninsured and underinsured patients to access (Hall et al., 2013).

A policy option for increasing health insurance affordability among employees is to allow employees to opt-in to Affordable Care Act (ACA) marketplace plans with subsidies. Currently, if an employer's plan meets ACA affordability and coverage criteria, employees are ineligible for marketplace subsidies. Under the Federal Poverty Level safe harbor rule of the ACA, employer-sponsored coverage is considered affordable if at least one plan offering does not exceed $103.14 per month in premiums for single coverage (Brandon, 2021). It is vital for social workers to advocate to change policies that do not allow marketplace subsidies for financially vulnerable populations.

Premiums are only one component of employees' cost share and many employees need family coverage which is much more expensive. The inability of employees to opt-in to ACA marketplace subsidies is particularly problematic for employees entering or reentering the workforce who experience lost Medicaid eligibility ($0 premium) and assume employer coverage ($500 premium, on average). Social workers can advocate for longer-term and less administratively complex policy solutions that allow individuals and families to opt-in to Medicare coverage, raising income eligibility thresholds for Medicaid, and making Medicaid a federal program only which provides states budgetary relief and protection against the volatility of health care expenditures (Brandon, 2021).

It is important to consider the implications of these types of policy changes with respect to the COVID-19 pandemic. Workers in our study already struggling to pay medical bills might have been reluctant to seek care if they had been exposed to the coronavirus (especially in customer-facing industries) and/or decided to still come to work to avoid losing pay, potentially putting themselves and their coworkers at risk. Nearly a third (29%) of workers with household income below $40,000 said they went to work with COVID-19 symptoms because they could not afford to lose pay (Kaiser Family Foundation, 2022a), suggesting the need for social workers to advocate for universal paid sick leave. Also, households in which a member lost a job at some point during the pandemic were nearly four times more likely to have problems affording medical care or prescription drugs compared to households with no job loss (Despard et al., 2021). Having the opportunity to opt-in to ACA Marketplace subsidies or Medicare would give workers uninterrupted health coverage in the event of job

loss. More research on this topic from the social work perspective is needed to protect individuals and families struggling to pay for medical expenses.

Conclusion

We illustrate how a well-intended effort to provide a technological solution to help employees enrolled in HDHPs pay for their healthcare expenses illuminated problems of underinsurance. Rather than solely focusing on increasing the overall rate of insurance, a standard of affordability should be applied that includes individuals and families' full cost share relative to their incomes and used to help assess the sufficiency of employer, health care provider, and public policies. Social workers are in an ideal position to advocate for these health care policy changes to protect financially vulnerable populations. Examples of advocacy activities include organizing meetings and educating health care staff, patients, and families about patient eligibility for charity care and creating petitions to improve the advertisement of charity care.

Note

1. A case included one or more bills.

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